



JAMAICA CONSTABULARY FORCE
Domestic Violence Training Manual Toolkit

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Spotlight Initiative
To eliminate violence
against women and girls

Initiated by the European Union and the United Nations:



JAMAICA CONSTABULARY FORCE DOMESTIC VIOLENCE **TRAINING** MANUAL TOOLKIT



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Acknowledgements

This domestic violence training manual toolkit for the Jamaica Constabulary Force (JCF) has been made possible under Pillar 4 of the EU-Funded Spotlight Initiative partnership with the United Nations Population Fund (UNFPA) to provide essential quality and accessible services to victims/survivors of gender-based violence. This toolkit of resources for activities is to be used alongside the training manual.

Special thanks to the Central Security and Safety Branch (CSSB) of the Jamaica Constabulary Force (JCF), National Police College of Jamaica (NPCJ) and the United Nations Population Fund (UNFPA) for their individual and collective contributions to the creation of the manual which was also guided by facilitation sessions of the 5-week domestic violence training of trainers course.

Special writing contributions from Elga Salvador (UNFPA Focal Point) and DSP Jacqueline Dillon (Head of the Domestic Violence Unit of the CSSB) and Kyisha Patterson (Consultant).

Acronyms and Abbreviations

BGA	Bureau of Gender Affairs
CPFSA	Child Protection and Family Services Agency (Child Development Agency)
CSO	Civil Society Organisations
CSSB	Community Safety and Security Branch
DV	Domestic Violence
DVA	Domestic Violence Act
DVICARE	Domestic Violence Intervention Centre
DVToT	Domestic Violence Training of Trainers
ESP	Essential Services Package
EU	European Union
EWARS	Early Warning Awareness and Response System
GBV	Gender-Based Violence
IPV	Intimate-Partner Violence
JCF	Jamaica Constabulary Force
JDF	Jamaica Defence Force
MCGES	Ministry of Culture, Gender, Entertainment and Sport
MoHW	Ministry of Health and Wellness
NCD	Non-Communicable Diseases
NGO	Non-Governmental Organisation
NPCJ	National Police College Jamaica
OCA	Office of the Children's Advocate
SI	Spotlight Initiative
SOP	Standard Operating Procedure
ToT	Training of Trainers
UNFPA	United Nations Population Fund
WHO	World Health Organisation

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Please note that the pre- and post-tests that will be compiled will not be used for evaluating your skills as a provider or member of your organisation, and to guarantee this we are keeping it confidential (we are not asking for your personal information).

The cumulative results of these tests will be used by the Spotlight Initiative for the reporting purposes and to self-evaluate the workshop based on the change in knowledge, to analyse which themes would need to be further strengthened.

Please kindly answer the following questions without any external help or references, based on your experience and knowledge of the topics at hand.

We really appreciate your contributions.

Thank you.

1. The term gender refer to:
 - a. Biological differences defined by chromosomes and genitalia.
 - b. The emotional, romantic, and sexual orientation of a specific group of people.
 - c. A reference to the socially constructed roles of women, men, girls and boys.
2. What are the root causes of GBV? (Check all that apply among the options below)
 - a. Alcohol/drug abuse
 - b. Gender Inequality
 - c. Extreme Poverty
 - d. Power Imbalance
 - e. High Levels of Stress
 - f. Education (or lack of education)
 - g. Learned Behaviours
3. GBV only affects women and girls
 - a. Yes
 - b. No
4. Which are the different types of gender-based violence
 - a. Physical
 - b. Physical, Sexual and Domestic Violence
 - c. Physical, Sexual and Intimate Partner Violence
 - d. Physical, sexual, psychological, and socio-economic

Pre-Test GBV- ES (Answers)

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CASE STUDIES

- CS1a - Scenarios with Reflection - ES [CS1a-GBV Scenarios with REFLECTION-ES.docx](#)
- CS1b - Scenarios without Reflection - ES [CS1b-GBV Scenarios WITHOUT REFLECTION-ES.docx](#)

CS2 - Case Study_DONAVAN and TERRY NEWBY

Donavan and Terry met at a birthday party for an associate of Donovan in 2015. They immediately developed an interest in each other and over a 2 year period, they went out on numerous dates and attended special events together organised by both their companies.

In June 2018, Donovan and Terry decided that they wanted to be with each other and committed to taking the next step in their relationship. They got married in August of the same year at an elaborate hotel on the North Coast in Jamaica.

On the night of their honeymoon, Donovan wanted to cement their marriage but Terry claimed she was not feeling well and wanted to sleep. Donovan became very frustrated as this was not the first time Terry was blowing him off when it came to sex. About 5am the next morning whilst Terry was asleep, Donovan began to caress her in her sleep and eventually made love to her. Terry woke up during the intercourse and attempted to fight him off but Donovan was too strong.

Things quickly went downhill from there and the violence moved from emotional to physical abuse. Terry threatened to report Donovan to the police for raping her. Terry told her mother of her ordeal who advised her to seek help. She told her to speak to the police. She told her mother that she loved Donovan and did not want him arrested. She stated that she fears for her life as Donovan is well connected and can do her and her family harm if he so desires.

Task:

**Terry came to your station and sought your help to deal with the matter.
You are to:**

- (a) Advise her on how to get help**
- (b) Terry wants to get a Protection Order, guide her on the process**
- (c) Conduct a Risk Assessment on Terry based on her report**

CS3a - Case Study_BRUCE AND ANNIQUE

Annique, a sergeant of Police and Bruce have been together for 10 years and have been married for 7 years. They do not have any children in the union however Annique has a child from a previous marriage. Her daughter is now 16 years old and resides in the household.

Things have been rocky with them for the entire period of their marriage as Annique often becomes violent with her husband. Her Husband holds a high profile position in government and is restrained in his reaction to both her verbal and physical abuse because reporting to the police has gotten him nowhere.

Annique during her childhood years has been a victim of emotional and physical violence at the hands of family members and has been sexually assaulted by male members in her family. In order to cope, she developed, over the years, physical strength through weight lifting and bodybuilding as she vowed never to allow any man to abuse her again. These experiences as a child caused mental health issues to Annique who often becomes violent if she feels threatened by any action of her husband verbal or otherwise. Bruce has never physically abused her however they often exchange spiteful words.

One day Bruce, having attended an office engagement out of town, got home at 12 midnight to find him being locked out of the house, as Annique had changed the locks earlier in the day. Bruce begged Annique to let him inside the house but Annique refused stating that Bruce is married and should not be home so late. She accused him of having an extra-marital affair.

Bruce called the police and only then did Annique let him inside the house. Once inside the house she hit him with pillows and began to throw his clothes outside the bedroom. Annique became further enraged when Bruce told her she gave him no reason to want to come home.

She then used a belt buckle to him in the eye. The police officer who responded to the call witnessed the entire situation and threatened to arrest Annique immediately but became sympathetic when he realised Annique was his colleague.

The responding Police Inspector took her aside and spoke with her outside of Bruce's hearing. She then left and locked herself in her bedroom. She told the police that if they come any further she will kill herself as she has bottles of gasoline inside her room to do so. This is not the first time Annique has threatened to kill herself if Bruce involved the police and embarrasses her. Three years prior after a similar incident, She took a bottle of pills after Bruce told her he was going to the police after Annique kicked him in the groin.

Bruce eventually left the home on advice of the police and has not been back. Annique has since filed several complaints against Bruce even though he has not been back to the house since the last incident.

Question:

The responding Police Inspector spoke to you in confidence and requested help. Bruce has taken to social media to talk about the police non-action.

How can you help Annique and Bruce?

Use all the processes learnt to assist you.

Present your findings and suggestions for action in a presentation.

CS3b - Case Study_BRUCE and ANNIQUE (DVTtoT)

Annique, a sergeant of Police and Bruce have been together for 10 years and have been married for 7 years. They do not have any children in the union however Annique has a child from a previous marriage. Her daughter is now 16 years old and resides in the household.

Things have been rocky with them for the entire period of their marriage as Annique often becomes violent with her husband. Her Husband holds a high profile position in government and is restrained in his reaction to both her verbal and physical abuse because reporting to the police has gotten him nowhere.

Annique during her childhood years has been a victim of emotional and physical violence at the hands of family members and has been sexually assaulted by male members in her family. In order to cope, she developed, over the years, physical strength through weight lifting and bodybuilding as she vowed never to allow any man to abuse her again. These experiences as a child caused mental health issues to Annique who often becomes violent if she feels threatened by any action of her husband verbal or otherwise. Bruce has never physically abused her however they often exchange spiteful words.

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Bruce eventually left the home on advice of the police and has not been back. Annique has since filed several complaints against Bruce even though he has not been back to the house since the last incident.

TASKS:

1. Analyse the scenario above; outline how the EIS Policy and any other policies can be used to address the situation outlined. Include in your answer, agency (ies) that can be contacted to help in the intervention and their individual role in the process.

[25

marks]

2. It is said that Gender-based Violence, more so Domestic Violence is deeply rooted and entrenched in the culture of many countries to include Jamaica. You were asked to present on this topic to a group of probationers. Develop a lesson outline, that includes, the background, aim, objectives, lesson outcomes and what method you would use to quality assure the presentation.

[25 marks]

CS4 - Case Study_JANE and RICHARD (DVToT)

Jane and Richard met at university and later married. They had two children together, and finally separated after 25 years. Both are tertiary educated, however Richard assumed the primary earner role early in the relationship and priority was given to his professional advancement. Jane supported Richard's career pursuits—which involved a number of relocations, here and overseas—and became the primary caregiver of their children who are in their mid to late teenage years. The children live with Jane and have limited

contact with Richard who continues to live and work overseas and returns periodically. There are no Family Court parenting orders, however there are consent orders dealing with the settlement of marital property between Jane and Richard and access to children during school holidays. Jane had completed a Graduate Diploma whilst raising the children, and had been in unrelated paid employment for brief periods when the family was based in Georgia. However she had not attained any significant experience or career progression as caring for the children was her main priority. Just prior to separation Jane accepted a full-time job that for the first time utilised her degree. She supports herself and the children from her modest income; and under the terms of the consent orders, Richard is discharged from any responsibility to pay child support. The property settlement entitles Jane to funds from the sale of a house property, which she intends to use to purchase a home for her and the children.

Marrying so young, Jane had little experience of relationships or what to expect. Richard always earned a high salary and from the outset took charge of their finances and purchases. As a couple they bought a number of real estate properties, which he referred to as his own, despite their being registered in Jane's name and clearly shared assets. Without the qualification and experience she'd hoped to obtain, Jane could only work in low-paid positions; and, after having children and living as expats overseas, was unable to work at all. Richard belittled Jane for her lack of financial contribution, and regularly monitored her weekly spending against the allowance he had allocated that was often insufficient for essentials. She was required to show Richard receipts for all of her expenditure. Richard could become enraged when he believed Jane had overspent or acted without his

approval and would often throw and break items precious to Jane to show his displeasure with her and to intimidate her. Early in the relationship when Jane freely expressed her opinions with Richard, he threw a coffee cup toward her head which flew out an open window and travelled 5 metres before smashing on the neighbour's brick wall. On another occasion he ripped up certificates of her achievement in music. If she went out, he would call her regularly to check on her movements and who she was with. Over time Jane tried to follow his instructions to avoid his angry outbursts; however, she found herself becoming increasingly isolated, anxious, and depressed, diminished by Richard's abuse, and lacking in self-esteem. Meanwhile, she was caring for two children, for the most part by herself, and coping with the additional challenges of establishing a home and friendships and raising a family in various overseas locations. Despite Jane's 'anything-for-peace' approach, Richard repeatedly criticised her mothering and homemaking abilities; and the children would often express concern for how she was feeling after Richard had finished his abusive rants.

Jane became distressed about how one of the children was coping and behaving and managed to get Richard's approval to spend money on therapy on the basis that it was about the child. It was during this time that Jane realised that she needed help herself.

Jane describes an emotionally traumatic separation that was prolonged over five years as she struggled to find ways to persuade Richard to address the problems in their relationship together. Jane felt she had to try hard to do what she could to save the marriage, but at the same time was learning to understand and deal with the fact that Richard had controlled her behaviour

for so many years. She decided that she needed help and was advised to contact the Police who can help.

Tasks:

- (1) Based on the scenario, develop a training plan to sensitise probationers in your division. Including a clear rationale, aim, learning objectives, methods of instruction delivery, learning outcomes and course structure.**

[25 Marks]

- (2) Discuss how the current legislations in Jamaica can be used to address the issues in this scenario. Cite any international laws that may be adopted in Jamaica to address issues like these.**

[25 marks]

CS5 - Case Study_John (Retired Police Officer)

John is 70 years old and a retired Police Officer who lives in a comfortable home in rural Jamaica with his son, George, George's Wife Lucy and 2 children ages 8 and 5 years. His wife Violet, died when George was 20 years old. Both George and Lucy are Police Officers.

John's health deteriorated two years ago following a heart attack. He had a programme of reablement for six weeks following his return from hospital and shortly afterwards his son George moved in to support him. John finds it tiring to do anything physical. His continuing heart problems mean he has become more and more dependent on George and Lucy over time, and has started to need

support with his personal care. His heart condition has resulted in him recently being diagnosed with vascular dementia. George and Lucy do most of the housework, gardening, shopping, cooking, cleaning and walking the dogs as well as taking care of their children.

George was shot and injured on the job. This has resulted in him experiencing lifelong disability including headaches, depression and mood swings. He has difficulties with his memory and attention and can be irritable and aggressive at times as a result he is placed on desk duty to update records. He has no direct contact with the public on the job most times, however because of shortage of staff has to do station guard duties sometimes. Apart from physical challenges George also faces financial difficulties and therefore moving in with his father also meant that the financial difficulties he was facing as a result of his rent increases were reduced.

John's pension and George and Lucy salaries provide them both with enough money to live on. John has suggested recently that he could bring in a care worker to do some of the jobs that George and Lucy are undertaking. George will not hear of this, insisting that they are fine and he doesn't mind looking after his dad. He became angry with John, asking why John couldn't have confidence in him 'like a father should in a son'. However, when he is angry he also complains that he doesn't have time to live his own life any more as there are too many responsibilities and no one he's taking his health into consideration.

John's General Practitioner (GP) has contacted his friend who is the Superintendent for the Parish because John reported that George has started to become violent towards him, his wife and the children. George has threatened violence and actually punched and kicked John on a number of occasions. John wants help for George. He also physically assaulted Lucy with a hot pot and locked her and the children outside the house on numerous occasions.

George's violence is also seen at work based on his treatment of his co-workers as well as members of the public he interacts with. His supervisors want help for him but do not want to go through the formal system.

Tasks:

Based on the JCF Early Intervention System and the SOP on Domestic Violence, assess the family situation, and initiate an intervention proceeding for the family.

- 1. Outline the likely issues the family faces and what are the risk factors if any for the children, John, Lucy and even George.**
- 2. As the Chaplain, what system can you recommend be put in place to help George and Lucy who are both Police Officers.**

CS6 - Case Study_Maria

Maria, 24, lives with her 15 month old daughter, Sophia, and Sophia's biological father, Graham (34) who is a Police Sergeant in Gloss Division. Maria came to live in Jamaica from London after meeting Graham online. Maria's teenage years were difficult as her father was emotionally and physically abusive towards her and her mother. Maria took a business studies course and was involved in the University of Technology Diving club.

In 2019, equipment failure during one of the dives caused Maria to sustain a spinal cord injury. Maria was rehabilitated from the hospital. With professional support she was able to regain much of her independence although she suffers from chronic pain as a result of the injury, and anxiety and depression related to this. She uses a wheelchair. Maria was advised that giving birth could further disable her but she was determined to have her baby for Graham as he is determined that he needs an heir.

Maria got pregnant and shortly after this she cancelled her contract with the domiciliary care agency. She said that Graham was able to do everything that she and the baby would need. Graham took time off work to be present at every ante-natal class. He works full time as a police officer and part-time doing security guard work. His shifts are often long and oftentimes he gets little sleep between jobs and taking care of Maria.

Graham has started to report for work late, is aggressive to colleagues and neighbours and has reported to the police that Graham is often heard threatening Maria and baby Sophia. There have also been reports to CPFSA about the family situation by concerned family members. The therapist that sees Maria is also concerned but is afraid to make an official report because of Graham.

There are concerns about Sophia's lack of development, low weight and being frozen in her emotional responses. Maria is reported to seem tired, wary and dishevelled and potentially in need of social care support to be an effective parent. The housing association had had complaints from neighbours about the family's dogs' waste covering the shared 'play' area and that they seemed dangerous. Graham's supervisor has reached out to the DVICARE unit for help.

Tasks:

- 1. What is your professional opinion about this case? What evidence is there of coercive control? What action is being taken and why?**
- 2. Use your knowledge in how to deal with Trauma in Domestic Violence to assist this family. Include in your presentation all the processes to be employed both by the DVICARE Manager and the Chaplains.**
- 3. Use an appropriate method to present your findings and explain the reason for each process.**

CS7 - Case Study_NORA (DVToT)

Nora is a 34-year-old woman with an immigrant background. She has lived here in your country for three years with her parents and sisters. Nora married Peter two years ago. Peter is the son of a family friend of Nora's parents. Nora's family comes from a patriarchal culture where the community comes before the individual.

Nora's marriage with Peter was a relief for Nora's family since in their culture a woman at Nora's age should not be single. However, quite soon after Nora and Peter got married, Peter started to control her everyday behaviour. Peter does not let Nora see her friends or go anywhere without him. A mandatory language course is the only place where Nora can go alone.

Peter takes away Nora's debit card and takes loans under her name. When Nora tries to resist, Peter turns violent and abuses her. Peter threatens to send Nora back to her home country.

Nora discloses the situation to her parents, asking for help. First, the parents take Peter's violent behaviour seriously, but suddenly Nora's father passes away. Nora's grieving mother is not capable of standing against Peter's will on her own.

At the same time, Peter spreads rumours about Nora's immorality in order to justify the claims of his violent actions to their community. The rumours humiliate Nora's family. The community pressures Nora's mother and her sisters' families to clear their name.

Nora's mother begs Nora to stay with Peter to calm the situation and her sisters ask her not to bother their mother with the issue any more. Nora feels that she is responsible for the violence and her family's reputation, and accepts that divorcing Peter is out of the question.

Over time, the violence gets more serious and more frequent. On one occasion, Peter strangles Nora for so long that she loses consciousness. After the strangulation, she starts to have speech impairment issues especially in stressful situations. Nora feels isolated, helpless and depressed.

Peter has threatened to share some private pictures of Nora in public if Nora ‘ruins his reputation as a husband’, as he puts it. Nora feels anxious since she cannot talk to anyone – even her family – about her feelings.

Tasks:

1. **Based on the scenario, you are to examine and analyse the types of violence identified. Outline the systems in place nationally that can be employed to treat the problem, bearing in mind the legislations, JCF policies, the help seeking process in the continuum of care including social services that can be used to assist Nora.**

[15 marks]

2. **Develop a safety plan for Nora and outline how this can be implemented (include conditions if Nora intends to leave, stay or has left)**

[5 marks]

3. **Peter has contacted his friend to beg Nora to come home. Identify, if any, the risk that Nora is likely to face even after intervention is done. Complete a Risk Assessment plan for Nora.**

[10 marks]

4. **Using the scenario as a guide, develop a lesson outline inclusive of a background and rationale to the topic “Impact of Domestic Violence on the Family”**

[20 marks]

[Total: 50 marks]

CS8 - Case Study_SAM STORY

Myra and I have been married for six years. I used to do everything I could for her. I paid off her debts, paid her bills and paid for her car. But I soon learnt that nothing would ever be enough for her. The emotional abuse started first. I was very rarely allowed to go anywhere by myself. When I was, it would only be to work and even then she would phone me constantly throughout the day. I tried to leave her when this started, but she emotionally blackmailed me to stay by overdosing on tablets; which I later found out she would spit them out under the bed. She would also threaten to hurt any future girlfriends I have so badly that I wouldn't want to be with her.

Optimistically thinking that things could get better between us I proposed to her after two years of being together. This was the biggest mistake of my life- things went dramatically downhill from that moment. The first time she viciously attacked me was on Good Friday 2021. I don't know why or what provoked her. She ran into the house, grabbed a knife and as soon as I walked in, she was attacking me with it. She then grabbed my testicles and twisted them as hard as she could and would not let go. It was excruciatingly painful. To this day I still do not know what caused her to be so violent. She would just snap from nice to nasty in an instant. The violence only got worse from there.

The second time she attacked me, she followed me around the house punching me in the head, hitting me with a pint glass, knocked *me* to the floor and proceeded to drop her knee into my head repeatedly. It was ferocious and I genuinely feared for my life. I also remember on another occasion she was punching me in the eye when I was driving around a roundabout, so hard that she bruised her knuckles. I was however later in the wrong for causing the bruising. The most shocking attack however, happened on our wedding night. She really beat me, kicking and punching

me repeatedly. I remember her digging her nails into my cheek, it felt like she was going to rip my cheek off. I managed to get away and ran down the road in bare feet and my wedding suit. I went back because she was threatening to hang herself with my wedding tie. I later got beaten because the cuts on my face ruined our honeymoon pictures.

I want to report to the Police but I am fearful of them not believing me. My wife is going to kill me if I stay.

TASKS:

Mary, Sam's neighbour, telephoned your station and spoke to you, the Station Commander seeking help.

1. Advise her and State your action

2. The report was taken and 5 days later you realise the DV referral form was still at your station. State your action

CS9 - Case Study_SARAH

Sarah is having her first baby. She's in her early 20s, and recently married: her parents and in-laws spent a lot of money on a lavish church wedding and honeymoon. On her wedding day, Sarah paused on her way into the church, her arm linked in her father's, feeling a dread in her gut that she was making the wrong decision in marrying this man but currently saw no way out; few narratives had shown her a life other than marriage to a man and children.

Several years earlier, after high school, Sarah got a job at a factory. There she noticed a young man paying particular attention to her, someone known around the factory as a nice guy, and she decided to go on a date with him, not having had any previous romantic encounters. The date started off well, and after dinner and a movie, he offered to drive her home. Instead of taking

her straight there, however, he took Sarah to a secluded area and demanded she have sex with him. When she said no, he did anyway, forcibly. After a derogatory comment about her loss of virginity, he dropped her off at her parent's home. Feeling overwhelmingly ashamed and shocked, Sarah took a bath and contemplated taking a bottle of pills. The next day, she told her mother what happened, and her mother, fearing her daughter be labelled as promiscuous, urged that Sarah marry him and make the best of the situation.

Caring deeply about her family's approval, Sarah got married despite a sense of dread. She became pregnant shortly after her wedding, and her husband also began hitting her around this time. Feeling an overwhelming instinct to protect her child, Sarah did everything in her power to placate her husband, including enabling his drinking to the point he would pass out and would no longer be a threat. Throughout her pregnancy, her husband became increasingly violent, though he avoided causing visible evidence, and became obsessed with the possibility that this wasn't his child, and that Sarah had cheated on him.

TASKS:

Sarah was encouraged to seek help from the police to protect her and her baby. She visited the Darlington Police and Spoke with Sgt. Donothing. However she was directed to you, the Sub-officer in charge, after Sgt. Donothing sent her home telling her it is not a police matter.

- 1. State your action and advice to Sarah**
- 2. What action if any can you take against Sgt Donothing based on her response to Sarah.**

CS10 - Case Study_FRANCIS & MARK

Francis and Mark were together for 23 years. Francis has been significantly hearing impaired since birth and wears hearing aids. She grew up in a loving but strict family environment, and met Mark when she was still a teenager, having had little experience with intimate relationships or independent living. They both completed year 10. Francis has limited TAFE qualifications and has worked periodically throughout the relationship when her child rearing responsibilities permitted; Mark ran his own one-man business for a time. For a number of years their income was derived predominantly from social security benefits. Mark has a history of misuse of alcohol and drugs, however Francis observed that he had developed ways of minimising its influence. The couple has three children at separation.

Mark began controlling and demeaning Francis early in the relationship. He became verbally abusive and aggressive when she was planning to go out with friends, he called her a "slut", and would punch the walls or doors or damage household goods. Francis says she "would pay for [her outings] for a long, long time after". While Francis had few friends and had moved away from her home city and family to be with Mark, over time she decided a night out wasn't worth the humiliation and fear. And yet these things came to characterise her experience of the relationship over many years and were made worse by a pervading feeling of insecurity due to her poor hearing. She describes crying every day, despairing at her situation.

Francis had thought often about leaving the relationship, and would at times tell Mark that she wanted it to end, however Mark would express remorse for his behaviour and plead with her to stay. Francis says her main reason for continuing in the relationship was a growing fear of what Mark may do if she were to take steps to get away. It was also the reason Francis denied the occurrence of domestic and family violence to family, friends and police for so long. Mark became more violent towards Francis once they began

having children. His abuse would always build from a verbal rage to wanton household property damage that would sometimes result in physical injury to Francis. This was the repeating pattern, and for Francis the occasions were too numerous to fully recount. There were however some incidents that were so concerning to neighbours that they called the police, but Francis felt too frightened to disclose the details of the violence knowing that Mark was nearby and likely to retaliate. Instead, she made up an account to shield the reality of the violence. Francis recalls that one night she locked herself and the children in the bathroom, and Mark punched the door in and smeared blood across the wall, in a rage about having to cook dinner.

Francis told police Mark had thrown a saucepan and didn't show them the blood or damage to the bathroom; she had tried to ring Mark's parents but couldn't go ahead with the call because she was worried her voice may be too loud and Mark would hear her. Police offered Francis little or no opportunity to make a proper statement and blamed her for fighting with Mark in front of the children.

On another occasion, when the couple was out with the children, Mark and his friends tried to pressure Francis into taking drugs, which she had never done or been prone to. Mark began calling her names, and on the way home he smashed the car interior while Francis drove. Once home, Mark damaged the guard at the front of the car and punched the laundry wall so violently he broke his hand. The following day Francis told him she would leave, but he pleaded with her not to and promised a special holiday, which never happened.

Not long after, Mark was arrested on charges unrelated to violence at home, of which he was later convicted. Although he avoided imprisonment, Francis believes, to some extent this was due to a favourable reference she felt she was pressured by Mark and his lawyer to provide to the court. Due to the nature of the charges Francis changed to part-time work so she could be with the children outside school and daycare hours, and continued to put up with Mark's violence and abuse. Later, Mark was charged with another serious offence. Pending his trial, a child protection order was issued

requiring that Mark move out of the family home and that he have no contact with the children for several months. Mark reacted angrily to these conditions, repeatedly demanding to see the children and continuing the violence.

Following another violent incident that involved Mark hitting one of the children, Francis told Mark to leave the holiday house the family were renting and get counselling. Initially, Mark complied. Four months later, after he was acquitted, he returned to the family home at midnight without Francis's consent, attacked Francis, and tried to throw her off the upper storey of the house when their young son physically intervened. Francis threatened to call the police but Mark pursued her around the house while the youngest child became more and more distressed. By this stage, Francis could see that the two older children were profoundly affected by their long exposure to the violence. Francis also discovered that Mark had access to a gun, and he began making threats to shoot her and a police officer.

TASKS:

Present your answers to the following questions using any form of aid to assist you in getting your point across.

- 1) Conduct a risk assessment and advise Francis on how to use the law for protection or otherwise.
- 2) Whilst speaking to Francis your own emotions came into play. State your action on how you would proceed.
- 3) What risks, if any, are the children facing and state how you can mitigate these risks.
- 4) What are the essential services that you would engage to assist in the intervention.

CS11 - Case Study - Jennifer (Referral Pathway)

Jennifer has been waiting with her sister Meg in the emergency room. Both show signs of physical trauma. Jennifer came home to find her partner, Jeff, locked in Meg's bedroom, with Meg calling for help. This started a chain of events that ended with both girls nursing bruises, and the imprint of Jeff's rings on their cheeks. Jennifer is angry with Jeff, but she doesn't know what to think of Meg.

Was her sister actually having an affair with Jeff?

How could she, knowing that Jeff was her main source of support for their twins and the house they all lived in belonged to him?

Meg was too scared to feel angry. The bruises would go quickly, but if Jeff ever sold her nude pictures online like he had threatened, her reputation might be ruined forever. That was the only reason she had allowed him back into her bed. At 15, she felt too young to be dealing with this.

CS12 - Case Study - Amanda (Referral Pathway)

Amanda, 15, has shown up to the emergency room with severe bruises to her arms, torso and legs. Her parents are accompanying her. She is tearful and reluctant to give any account of how she received the bruises. Her mother, who is standing immediately next to her, is soft-spoken, timid and rarely speaks up or makes eye contact when questions are asked. Her father, on the other hand, is very outspoken and seems a bit impatient with the process. When questions are asked about Amanda's medical history her father shuts down the conversation immediately stating that he does not see the need for medical history to treat the bruises and demands that neither Amanda nor her mother answer any of the questions. Shortly after, he steps outside to take an important business call and Amanda uses the opportunity to whisper to the nurse that she does not want to go back home with her father. She also asks if there are any police officers stationed at the hospital. Her mother at this point is sitting in the lobby and therefore does not hear the conversation.

How should you treat this scenario?

CS13 - Case Study - Barbara & Devon (Referral Pathway)

The police have received a call that a domestic dispute at Lot 38, Pebbles Drive is getting out of control. This is the third time they have been called to that address in the space of two weeks. In the last two instances the alleged victim refused to give a statement against the alleged perpetrator. Upon arrival, Barbara is seen standing outside with blood coming from an open wound on her face. Devon, seemingly intoxicated, is at the doorway shouting verbal and derogatory insults at Barbara and throwing what seems to be her clothing outside on the lawn. Barbara seems to be scared and is inconsolable, therefore unable to speak clearly. Barbara is the sole provider of the household at the moment as Devon recently got laid off, however the house is owned by Devon. Two small children are inside the house behind Devon looking on in horror. Barbara has nowhere to go as all her family members have either migrated or are living in the rural area of another parish far away.

How should you deal with this scenario?

CS14 - Case Study - Monica (Referral Pathway)

Monica is a 28 year old house-wife who lives with her husband and in-laws at their family house. She is not Jamaican but moved to Jamaica 5 years ago when her long-distance relationship with Max began to get serious. English is not her first language and she still struggles to communicate fluently at times. She recently gave birth to a beautiful baby girl who is now 8 months old. In the beginning of their relationship Max, now her husband, was a charming and loving man, however, after she moved to Jamaica permanently and into the family house he became very abusive towards her. Her in-laws constantly tell her she should be grateful that their son has taken her in and has gotten married to her. Max has a well paying job and Monica has nothing financially to worry about, however, Max's aggression has been increasing and he now hits her every night. Some nights are worse than others. Last night he hit her in her eye and proceeded to beat her in the head with the iron because she had not washed and ironed his favourite work shirt. The baby was on the bed next to them but he did not seem to care.

Monica recognizes that her and her child are not safe but she is afraid to leave because Max is a licensed firearm holder and she is unable to predict his reaction. He has taken away her passport and other IDs and travel documents and threatens that if she tries to leave he will take her child away from her and shoot her before she has a chance to leave the island. Her friend had told her of a GBV helpline that she could call for help. She goes to the backyard where she can get some privacy and hides to make a call to the helpline.

How will Monica be supported to leave this situation safely with her child?

CS15- Case study_Mary

(Safety Planning)

EXERCISE 1

Mary has been in an abusive relationship for over 15 years. She has two children with her husband, ages 12 and 9. She is beaten by her husband almost daily. The children are often in their father's wrath. Mary has made many reports to the police and is in possession of a copy of a protection order served on her husband. She also has a safety plan that she developed. Every Saturday, Mary and her children go to the market and supermarket to buy food. She has reached out to you as a first responder to assist her to leave.

Outline your strategy to help her get to safety.

EXERCISE 2

Mary's husband was arrested by the police for assault. She is to go to court. She needs assistance to get to court from her safe location. She has reached out to you, the centre/shelter manager, to assist her.

Outline how you would facilitate this request.

Is it safe to take Mary to court in your personal vehicle if no other is available? Discuss

RG1a- Leave No One Behind (LNOB) Game Characters
Character LNOB GAME
HIV+ man
HIV+ woman
Trans Woman
Trans Man
Lesbian
Gay
Adolescent girl victim of rape
Adolescent boy victim of rape.
Old woman with visual impairment.

Wealthy Woman.
Wealthy Man.
Illiterate adult woman who only speaks patois.
Illiterate adult man who only speaks patois.
Woman sex worker
Man sex worker.
Trans sex worker.
Victim of human trafficking.
Woman with multiple partners.
Man with multiple partners.
A poor man
A poor woman
A teen girl

A teen boy
A pregnant teenager
A woman inmate
A man inmate
An adolescent mother
A pregnant adolescent
A woman from rural area
A man from a rural area
A person with a mental disability
A person with a physical motorial disability

RG1b-Leave No One Behind (LNOB) Game Instructions

“Leave no one behind” role playing game

Objective of the activity:

This activity permits us to identify obstacles based on gender biases and stereotypes faced by men, women, girls, boys, and marginalised populations of both sexes.

Preparation:

prepare a set of subjects and a set of statements.

Set of subjects, an example:

- HIV+ man/HIV+ men/HIV+ transgender;
- Transsexual Woman/Transsexual Man;
- Lesbian/Gay;
- Adolescent girl victim of rape/Adolescent boy victim of rape;
- Old man with a physical disability;
- Old woman with visual impairment;
- Wealthy Woman/Wealthy Man;
- Illiterate adult woman who only speaks patois;
- Transvestite;
- Woman sex worker/Man sex worker; Transgender sex worker;
- Victim of human trafficking;
- Woman with multiple partners; Man with multiple partners
- A poor man
- A poor woman
- A Girl
- A boy
- An adolescent mother
- A woman from rural area
- A man from a rural area

be sure to have at least a set of subjects that permits you to distribute a different subject per each participant; beforehand prioritise which subjects you want necessarily to use among the most marginalised populations; to show the gaps we have to keep at least one or two subject/s who usually do not suffer discrimination.

Set of statements, an *examples* follow (we can adapt them to UNFPA's priorities and mandate of the NBFP):

- *“I feel comfortable to go to the health service to report a sexual violation”*
- *“I feel providers listen to me and give me proper information when I am telling them I have been victim of physical violence”*
- *“I feel comfortable to go to the health service and ask for a condom”*
- *“I can easily reach the health service at the third floor”*
- *“I find it useful to receive informational material with a lot of text in English”*
- *“I always feel welcomed and treated with respect and dignity at the police station”*
- *“I feel comfortable to go to the health service with my partner”*

- *"I have no problem in going to the Court to report my ex who physically and sexually abused me"*
- *"I feel comfortable sitting in the waiting room of any service provider, people treat me kindly"*

Methodology:

Each participant is given a cardboard card with printed in it a different subject he/she has to identify with (see above the possible subjects).

All participants are in a line close to the wall of the room. They are holding the cardboard card in their hands not showing it to their colleagues and have 1 minute of time to identify themselves with the subject written in their specific card.

The facilitator explains that he/she will read some statements. After listening to each statement each participant should think if this statement is true or false for the identity he assumed in this role-play. If the statement is true for his/her game-identity, the participant should make a step forward. If the statement is false for the identity he/she assumed, the participant should stay where he/she is.

When all participants are ready and confirm they understood, the facilitator can start reading some (approx. 20) statements.

Participants have to step forward if they consider that the person with the identity they represent would say "yes". They stay behind otherwise.

At the end of the game a discussion will be raised, people will say why they were left behind, others will say why they could step forward. Each participant will present the situations they faced being the subject in their card and will express the obstacles, difficulties and discrimination they faced. This will raise biases from health providers, obstacles related with physical structure and other characteristics that limit the accessibility to a high quality health for different populations.

Stress has to be put in the idea that everyone should have reached the same distance, since "The enjoyment of the highest attainable standard of health is one of the fundamental rights of every human being", this should be granted, without distinction of race, religion, political belief, economic or social condition, gender, sexual orientation, health status including HIV+, disability,

Leave no one behind is a fundamental principle of the Agenda 2030. So, all service providers have to assure that the fundamental human right to health is respected and have to offer services with quality and no discrimination to everyone, without discrimination.

To make things easier and the analysis more in depth, the cardboard cards could be distributed to participants when they register (inside an envelope or closed with a tape – asking them not to open it).

You can assign a person with the role of observer (who knows since the beginning who represents each subject) to take note of how many steps each participant does and what each participant says and make a final analysis.

RG1c-Leave No One Behind (LNOB) Game Sentences

Sentences LNOB Game

- *“I know where to ask for support if I am victim of GBV”*
- *“I feel comfortable to go to the health service and ask for a condom.”*
- *“I feel comfortable to go to the health service to report having just experienced a sexual violation.”*
- *“I feel providers listen to me and give me proper information when I am telling them I have been victim of physical violence.”*
- *“I always receive sexual and reproductive health services free of biases.”*
- *“I can easily reach the health service at the third floor.”*
- *“I find it useful to receive informational material with a lot of text in English.”*
- *“I always feel welcomed and treated with respect and dignity at the police station”*
- *“I am sure I will be treated with respect and dignity by the police if I should go to report having experienced sexual violence”*
- *“I feel comfortable to bring my ex-partner who sexually and physically abused me to the Court since I trust I will be treated fairly by the justice system.”*
- *“I feel comfortable to go to the health service with my partner.”*
- *“I feel comfortable sitting in the waiting room of any service provider, people treat me kindly”*
- *“I feel I will always be treated with respect service providers of the health, police, justice and social service sector”*

**RG2 - Mary and John - ES [RG2 - Mary and John - ES.pdf](#) TO BE PLACED
HERE**

RG3a- Referral_String_Character_Cards

**This exercise is taken from WHO Caring for
women subjected to violence: a WHO curriculum
for health care workers. Exercise 7.1
It is also used in the UNFPA's Clinical
Management of Rape package**

Kamile

You are Kamile, a 28 year-old woman who has been experiencing physical and sexual abuse from her husband for the last 6 months. You don't know what to do therefore you go to your sister for support and advice.

Sister

Listen to Kamile, talk to her and then send Kamile to the community/religious leader for advice

Community/religious leader

Listen to Kamile, talk to her and then send Kamile to the Women's group for support

Women's group

Listen to Kamile, talk to her and then send Kamile to the doctor/health clinic for health care

Doctor/ Clinic #1

Listen to Kamile, talk to her and then send Kamile to the police #1 for official medical reporting form

Police #1

Listen to Kamile, talk to her and then send Kamile back to the doctor/clinic #2 (doctor 1 is not available). Explain that no charges can be filed without medical proof

Doctor/ Clinic #2

Listen to Kamile, talk to her, take her history, perform a medical examination and then send Kamile back to the police #2 with medical details

Police #2

Listen to Kamile, talk to her and then send Kamile to legal aid

Legal Aid / Lawyer

Ask Kamile to tell her story, ask for all the documents, prepare her case and make her practice saying her story in court again

Court

Welcome Kamile and instruct her to re-tell her story at court

RG3b- Referral_String_Facilitator's_Note-ES

Referral string exercise: facilitator instructions

Duration

75 minutes

Objectives

- Increase awareness of and empathy for the difficulties women who experience violence face when seeking support.
- Highlight how gender norms can affect women's ability to seek help and access care.
- Encourage participants to think about what they can do as providers to provide an empathetic response to survivors of violence

Instructions

1. Ensure you prepared -in advance- a knitting string
2. Ask the participants to volunteer to play the characters of the referral string exercise
3. Assign one character to each volunteer, instruct them to read their roles
4. Start the role play by the main Character Kamile telling her story to her sister who refer her to the community leader, eventually starting many steps of referral -from one person to another-and telling her story many times to many people
5. While referring Kamile to service providers a string (handed for the first time by Kamile) should indicate the referral paths. You will end up with a nest of threads representing a complicated referral pathway
6. Ask
 - Kamile, how she felt
 - Other characters, how they felt
 - Observers, how they felt
6. Ask the participants:
 - How do you feel about the survivor's options for help and about the choices she was able to make and the process to get the support?
 - Probe: Was she always free to make the decision and access the services?
 - Probe: How much power did she have?
 - Probe: what do you feel about Kamile telling her story to many people and the complicated referral process?

- What made it difficult for the survivor to reveal her experience of violence?
- Do you think we respect the guiding principles of dealing with GBV survivors?

Wrap up and take away messages for this session

1. Referral string exercise illustrates the challenging decisions that women face in handling violence and how this is connected to how survivors respond to them, the information they provide and how the referral mechanism is put in place.
2. Women often have very few, if any, options for seeking support or escaping from violent situations. Many factors may prevent a woman from getting needed help from different sectors, including limited information, limited support, limited empathy.
3. Although it is important to provide the needed services in one centre to minimise traumatization and improve privacy, confidentiality and safety of the survivor, however, this is not available in most settings. **It is crucial therefore to develop a referral pathway with other service providers that respect and respond to guiding principles.**

Note to the facilitator: This exercise is designed for 10 participants, but other actors can be added and it can be contextualised to different scenarios.

These instructions of the exercise are taken from the UNFPA 2023 CMR package.

Domestic Violence Training Agenda - Two Day Workshop (Day 1 of 2)

Time	Minutes	Sessions	Main Subtopics
8.30-9.00am	30'	Registration	Attendance Recording, Devotion
9.00 – 10.30am	90'	Introduction to Domestic Violence (DV)	Definition of Domestic Violence (DV) Root Causes and Contributing Factors of DV Types of DV
10.30- 10:45am	15'	BREAK	BREAK
10:45 – 12.00pm	75'	Impact of Domestic Violence	Individual Children Family Community
12.00 – 1.00pm	60'	LUNCH	LUNCH
1.00 - 2:30pm	90'	Survivor Centered Communication Skills	Gender-Based Violence Guiding Principles Verbal and Non-Verbal Communication Active Listening Empathy vs. Sympathy
2:30 - 3:30pm	60'	Police Role in Domestic Violence	JCF SOP on DV Domestic Violence Act Force Orders
3:30 - 4:00 pm	30'	Group Exercise/Reflection	

Domestic Violence Training - Two Day Workshop (Day 2 of 2)

Time	Minutes	Sessions	Main Subtopics
8.30-9.00am	30'	Registration	Attendance Recording, Devotion.
9.00 – 9.20am	20'	RECAP	Guided Discussion Q &A
9.20- 10:30am	70'	Interviewing Techniques	Techniques in interviewing victims; witnesses & perpetrators Interviewing Children Recording incidents of Domestic Violence
10.30 - 10:45	15'	BREAK	BREAK
10.45 – 12.00pm	75'	Safety Planning	Danger to Safety Protocol Risk Assessment Role of DVICare Centre in Safety Planning
12.00 - 1.00pm	60'	LUNCH	LUNCH
1.00 - 2.30pm	90'	Referral Pathway & Protocols	Intersectoral response to DV Continuum of Care Warm referrals
2.30 - 3.30pm	60'	Group Exercise	
3.30-4.00pm	30'	Wrap Up	

Domestic Violence Training - Three Day Workshop (Day 1 of 3)

Time	Minutes	Sessions	Main Subtopics
8.30-9.00am	30'	Registration	Attendance Recording, Devotion.
9.00 – 10.30am	90'	Introduction to Domestic Violence (DV)	Definition of Domestic Violence (DV) Root Causes and Contributing Factors of DV Types of Domestic Violence (DV)
10.30- 10:45am	15'	BREAK	BREAK
10:45 – 12.00pm	75'	Impact of Domestic Violence (DV)	Specific impact on the following groups: Individual; Children; Family & Community Trauma and Grief
12.00 – 1.00pm	60'	LUNCH	LUNCH
1:00 pm- 2:20pm	80'	Survivor Centered Approach and Gender-Based Violence Guiding Principles	What is Gender-Based Violence (GBV)? What is the Survivor-Centered Approach? Four GBV Guiding Principles Intersectional Approach to GBV
2:20 pm - 2:30pm	10'	BREAK	BREAK
2:30pm - 3:30pm	60'	Survivor-Centered Communication Skills	Verbal and Non-Verbal Communication Active Listening Body Language/Verbal Messages Empathy vs. Sympathy LIVES Approach
3:30pm - 4:00 pm	30'	Group Exercise/Reflection	

Domestic Violence Training - Three Day Workshop (Day 2 of 3)

Time	Minutes	Sessions	Main Subtopics
8.30-9.00am	30'	Registration	Attendance Recording, Devotion.
9.00 – 9.30am	30'	RECAP	Guided Discussion/ Q&A
9.30- 10:30am	60'	Psychology of Self	What is self-concept? Effective use of self Active Listening Hearing vs Listening Process of Decision Making
10.30 - 10:45	15'	BREAK	BREAK
10.45 – 12.00pm	75'	Emotional Intelligence	What is Emotional Intelligence? EQ vs IQ Self-Awareness Motivation & Empathy
12.00 - 1.00pm	60'	LUNCH	LUNCH
1.00 - 2.20pm	80'	Role of Police in Domestic Violence	JCF SOP on DV Domestic Violence Act (DVA) Force Orders Police response in cases of DV
2:20 - 2:30 pm	10'	BREAK	BREAK
2.30 - 3.30pm	60'	Group Exercise	
3.30-4.00pm	30'	Reflection	

Domestic Violence Training - Three Day Workshop (Day 3 of 3)

Time	Minutes	Sessions	Details
8.30-9.00am	30'	Registration	Attendance Recording, Devotion.
9:00-9:30 am	30'	RECAP	Guided Discussion/ Q&A
9.30 – 10.30am	60'	Interviewing Techniques	Techniques in interviewing victims; witnesses & perpetrators Interviewing Children Recording incidents of Domestic Violence
10.30- 10:45am	15'	BREAK	BREAK
10:45 – 12.00pm	75'	Safety Planning	Danger to Safety Protocol Risk Assessment Role of DVICare Centre in Safety planning
12.00 – 1.00pm	60'	LUNCH	
1.00 - 2:00pm	60'	Referral Pathway & Protocols	Intersectoral response to DV Continuum of Care Warm referrals
2:00 - 3:30pm	90'	Case Study Presentations	Assessment
3:30 - 4:00 pm	30'	Wrap-Up	

Domestic Violence Training Agenda - Five Day Workshop (Day 1 of 5)

Time	Minutes	Sessions	Main Subtopics
8.30-9.00am	30'	Registration	Attendance Recording, Devotion
9.00 – 10.30am	90'	Introduction to Domestic Violence (DV)	Definition of Domestic Violence (DV) Root Causes and Contributing Factors of DV Types of Domestic Violence (DV)
10.30- 10:45am	15'	BREAK	BREAK
10:45 – 12.00pm	75'	Impact of Domestic Violence (DV)	Specific impact on the following groups: Individual; Children; Family & Community Trauma and Grief
12.00 – 1.00pm	60'	LUNCH	LUNCH
1.00 - 2:00pm	60'	Survivor Centered Approach and Gender-Based Violence Guiding Principles	What is Gender-Based Violence (GBV)? What is the Survivor-Centered Approach? Four GBV Guiding Principles Intersectional Approach to GBV
2:00 - 3:20pm	80'	Survivor-Centered Communication Skills	Verbal and Non-Verbal Communication Active Listening Body Language/Verbal Messages Empathy vs. Sympathy LIVES Approach
3:20 - 3:30pm	10'	BREAK	BREAK
3:30 - 4:00 pm	30'	Group Exercise/Reflection	

Domestic Violence Training - Five Day Workshop (Day 2 of 5)

Time	Minutes	Sessions	Main Subtopics
8.30-9.00am	30'	Registration	Attendance Recording, Devotion.
9.00 – 9.30am	30'	RECAP	
9.30- 10:30am	60'	Psychology of Self	What is self-concept? Effective use of self Active Listening Hearing vs Listening Process of Decision Making
10.30 - 10:45	15'	BREAK	BREAK
10.45 – 12.00pm	75'	Emotional Intelligence	What is Emotional Intelligence? EQ vs IQ Self-Awareness Motivation & Empathy
12.00 - 1.00pm	60'	LUNCH	LUNCH
1.00 - 2.20pm	80'	Role of Police in Domestic Violence	JCF SOP on DV Domestic Violence Act (DVA) Force Orders Police response in cases of DV
2:20-2:30pm	10'	BREAK	BREAK
2.30 - 3.30pm	60'	Group Exercise	
3.30-4.00pm	30'	Reflection	

Domestic Violence Training Agenda - Five Day Workshop (Day 3 of 5)

Time	Minutes	Sessions	Main Subtopics
8.30-9.00am	30'	Registration	Attendance Recording, Devotion
9:00-9:30 am	30'	RECAP	
9.00 – 10.30am	90'	Interviewing Techniques	Techniques in interviewing victims; witnesses & perpetrators Interviewing Children Recording incidents of Domestic Violence
10.30- 10:45am	15'	BREAK	BREAK
10:45 – 12.00pm	75'	Role of the DVI Care Centres	Functions of the DVICare Centres Locations of the DVICare Centres Operation/Protocols of DVICare Centres
12.00 – 1.00pm	60'	LUNCH	LUNCH
1.00 - 2:00pm	60'	Safety Planning	Danger to Safety Protocol Risk Assessment Role of DVICare Centre in Safety Planning
2:00 - 3:00pm	65'	Counselling & Mediation	Definitions of Counselling and Mediation Counselling process Mediation process
3:00- 3:10pm	10'	BREAK	BREAK
3:10 - 4:00 pm	50'	Group Exercise/Reflection	

Domestic Violence Training - Five Day Workshop (Day 4 of 5)

Time	Minutes	Sessions	Main Subtopics
8.30-9.00am	30'	Registration	Attendance Recording, Devotion.
9.00 – 9.30am	30'	RECAP	
9.30- 10:30am	60'	Domestic Violence and the Court	Orders that can be filled by domestic violence survivor Application of Orders
10.30 - 10:45	15'	BREAK	BREAK
10.45 – 12.00pm	75'	Referral Pathway & Protocols	Intersectoral response to DV Continuum of Care Warm referrals
12.00 - 1.00pm	60'	LUNCH	LUNCH
1.00 - 3.00pm	120'	Agencies Networking	Role and functions of specific agencies in relation to domestic violence Importance of regional/agency networking
3.00pm-4.00pm	60'	Case Study Presentations	Assessment

Domestic Violence Training Agenda - Five Day Workshop (Day 5 of 5)

Time	Minutes	Sessions	Main Subtopics
8.30-9.00am	30'	Registration	Attendance Recording, Devotion
9:00-9:30 am	30'	RECAP	
9.30 – 10.30am	60'	Case Study Presentations	Assessment
10.30- 10:45am	15'	BREAK	BREAK
10:45 – 12.00pm	75'	Simulation Exercises	
12.00 – 1.00pm	60'	LUNCH	LUNCH
1.00 - 2:30pm	90'	Simulation Exercises	
2:30 - 3:30pm	60'	Closing Exercise	
3:30 - 4:00 pm	30'	Wrap-Up	